QUORVANE BIOPHARMA, INC.

REQUEST FOR PROPOSAL

Phase III Clinical Trial — Oncology

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RFP Reference: QRV-2026-CALIBRATE-301	Issue Date: July 15, 2026
Protocol Number: QRV-CAL-301	Response Due: September 4, 2026
Compound: Veliforinib (QRV-4471)	RFP Contact: clinical.ops@quorvanebio.com
Indication: Advanced Hepatocellular Carcinoma	CRO Point of Contact: _______________

1. BACKGROUND AND STUDY RATIONALE

Quorvane Biopharma, Inc. ("Sponsor") is a clinical-stage oncology company focused on developing targeted therapies for patients with limited treatment options in hepatocellular carcinoma (HCC) and biliary tract cancers. Veliforinib (QRV-4471) is an oral, selective inhibitor of fibroblast growth factor receptor 4 (FGFR4) currently being evaluated in a Phase II dose-expansion study (QRV-CAL-201). Preliminary efficacy data from the Phase II study demonstrated an objective response rate (ORR) of 24.3% and median progression-free survival (mPFS) of 6.8 months in biomarker-selected patients (FGF19-amplified HCC), supporting advancement to a registrational Phase III trial.

HCC represents a significant unmet medical need globally, with approximately 905,000 new cases diagnosed annually. Despite approvals of sorafenib, lenvatinib, and atezolizumab plus bevacizumab in the first-line setting, prognosis remains poor with median overall survival of 12–19 months. The FGF19/FGFR4 signaling axis is amplified in approximately 30% of HCC patients, identifying a targetable molecular subgroup for whom veliforinib may offer meaningful clinical benefit.

2. STUDY OVERVIEW

2.1 Study Design

CALIBRATE-301 is a randomized, double-blind, placebo-controlled, multicenter Phase III study evaluating the efficacy and safety of veliforinib versus placebo in combination with standard-of-care atezolizumab plus bevacizumab in patients with FGF19-amplified, unresectable HCC who have received no prior systemic therapy.

Parameter	Details
Study Phase	Phase III, Registrational
Design	Randomized, double-blind, placebo-controlled
Randomization Ratio	2:1 (veliforinib + atezo/bev : placebo + atezo/bev)
Primary Endpoint	Overall Survival (OS) in FGF19-amplified population

Parameter	Details
Key Secondary Endpoints	PFS, ORR, DOR, HRQoL (EORTC QLQ-C30 + HCC-18)
Planned Enrollment	Approximately 480 patients (320 veliforinib : 160 placebo)
Target Population	Adults ≥18 years, FGF19-amplified HCC, ECOG PS 0-1, Child-Pugh A
Treatment Duration	Until disease progression, unacceptable toxicity, or withdrawal
Planned Study Duration	Approximately 48 months (enrollment 24 mo + follow-up 24 mo)
IND/CTA Status	IND active (FDA); CTA submissions planned Q1 2027

2.2 Countries and Sites

The Sponsor anticipates conducting CALIBRATE-301 across approximately 80–100 investigative sites in the following regions:

Region	Countries	Approx. Sites	Approx. Patients
North America	United States, Canada	20–25	100–120
Europe	Germany, France, Spain, Italy, Netherlands, Poland	25–30	120–140
Asia-Pacific	Japan, South Korea, Taiwan, Australia	20–25	140–160
Rest of World	Israel, Brazil	8–12	60–80

3. SCOPE OF SERVICES REQUESTED

The Sponsor is seeking a full-service CRO to manage CALIBRATE-301. The following services are included in scope. CROs are invited to propose alternative service models where they believe efficiencies or quality improvements can be achieved. All services must comply with ICH E6(R3) Good Clinical Practice guidelines.

3.1 Project Management

- Dedicated Global Project Manager and regional CTL oversight
- Risk-based project management framework with proactive escalation protocols
- Integrated project timeline management, milestone tracking, and RAID log
- Regular governance meetings (weekly operational, monthly steering committee)
- Budget management, change order processing, and financial reporting

3.2 Site Selection and Feasibility

- Global feasibility assessment across all planned regions

- Site identification leveraging existing oncology site networks, particularly HCC-experienced sites
- Regulatory landscape assessment for each country
- Site qualification visits (SQVs) and selection recommendations
- Investigator meeting planning and coordination

3.3 Clinical Monitoring

- Risk-based monitoring (RBM) strategy combining centralized and on-site monitoring
- Site Initiation Visits (SIVs), routine monitoring visits, and Close-Out Visits (COVs)
- Remote monitoring for data review and source data verification where applicable
- Protocol deviation management and CAPA implementation
- Real-time site performance dashboards accessible to Sponsor

3.4 Data Management

- EDC system build, validation, and maintenance (Sponsor preference: Medidata Rave or Veeva Vault EDC)
- CRF design, completion guidelines, and site training
- Data entry, cleaning, query management, and database lock activities
- Medical coding (MedDRA for AEs, WHO Drug for concomitant medications)
- Integration with central laboratory data feeds and ePRO platform

3.5 Biostatistics and Statistical Programming

- Statistical Analysis Plan (SAP) development in collaboration with Sponsor biostatistics team
- Interim analysis support (one planned IA at approximately 50% OS events)
- IDMC/DSMB support including data packages and meeting coordination
- Final statistical analysis and Tables, Listings, and Figures (TLFs)
- Integrated Summary of Safety (ISS) and Integrated Summary of Efficacy (ISE) support

3.6 Medical Writing

- Protocol and protocol amendment development (Sponsor-led, CRO support)
- Investigator Brochure updates (annual or as needed)
- Clinical Study Report (CSR) authoring for primary and secondary analyses
- Regulatory submission documents (Module 2.7 clinical summaries)
- Plain language summaries for patient and lay audiences

3.7 Pharmacovigilance and Safety Reporting

- SAE reconciliation between safety database and EDC
- Expedited regulatory reporting (IND safety reports, CTA safety reports) per regional requirements
- Support for DSMB charter development and safety data review packages
- Sponsor retains primary responsibility for PV database; CRO provides reconciliation and reporting support

3.8 Central Laboratory

- Central laboratory services or management of central lab vendor (TBD based on CRO recommendation)
- FGF19 amplification testing for patient eligibility screening (validated assay required)
- Pharmacokinetic and pharmacodynamic sample management
- Biomarker sample collection, processing, storage, and shipment logistics
- Laboratory manual development and site training

4. KEY ASSUMPTIONS FOR BUDGETING

The following assumptions have been used for RFP purposes. CROs should clearly state any deviations from these assumptions in their proposals. Changes to assumptions may require re-budgeting.

Enrollment: 480 patients randomized; screening failure rate estimated at 35%; ~726 patients screened

Enrollment Duration: 24 months from first patient first visit (FPFV) to last patient first visit (LPFV)

Site Activation: 80% of sites activated within 6 months of study startup

Protocol Amendments: Up to 2 substantial amendments assumed during study conduct

Monitoring Frequency: On-site monitoring approximately every 8–10 weeks per site during enrollment; risk-adapted during follow-up

Database Lock: Primary DBL within 6 months of primary OS analysis data cut

Regulatory Submissions: IND (US) is active; CTA filings required for all ex-US countries; timeline to first patient assumes parallel submissions

ePRO Platform: Sponsor will contract ePRO vendor directly; CRO to provide integration support and site training

Imaging: Central imaging review via Sponsor-contracted vendor; CRO to coordinate with imaging vendor

Investigational Product: IP supply and distribution managed by Sponsor; CRO to coordinate with Sponsor's clinical supply team

5. PROPOSAL REQUIREMENTS AND SUBMISSION INSTRUCTIONS

5.1 Required Proposal Sections

- Executive Summary (≤2 pages)
- Understanding of the Study and Sponsor Objectives
- Proposed Project Team: named individuals, CVs for key personnel (GPM, Medical Monitor, lead Biostatistician)
- CRO Experience: minimum 3 references for Phase III oncology studies completed in the past 5 years, including HCC or liver cancer experience where available
- Proposed Project Timeline with milestone dates (Gantt chart format)
- Risk Management Approach
- Technology and Systems: EDC, CTMS, eTMF platforms proposed
- Detailed Budget: by service area, by year, with FTE assumptions clearly stated
- Pass-Through Cost Assumptions
- Proposed Contract Structure (FSP, full-service, or hybrid with rationale)

5.2 Submission Deadline and Format

Proposals must be submitted electronically in PDF format to clinical.ops@quorvanebio.com no later than September 4, 2026 at 11:59 PM Eastern Time. File naming convention: [CRO Name]_QRV-CAL-301_RFP_Response.pdf. Proposals submitted after the deadline will not be considered. The Sponsor anticipates conducting CRO presentations during the week of September 14–18, 2026.

5.3 Clarification Questions

Written clarification questions must be submitted to clinical.ops@quorvanebio.com by August 14, 2026. Questions and responses will be distributed to all RFP recipients simultaneously to ensure a fair process. Verbal communication with Sponsor personnel regarding this RFP is not permitted during the evaluation period.

6. EVALUATION CRITERIA

Criterion	Weight
Scientific and operational understanding of HCC and FGFR4-targeted therapy	20%
Quality and experience of proposed project team (named individuals)	25%
Operational plan, timeline feasibility, and site network in target regions	20%
Technology infrastructure and data management capabilities	10%
Budget competitiveness and transparency of assumptions	15%
References and track record in Phase III oncology	10%